



The Corridor Outside Ward Three

Teacher copy (everything in the student copy, plus answers and teaching notes)

The article, marked for rhetorical devices (one color throughout):

1 I have memorized a corridor. Not a poem, not a prayer, a corridor: the third tile from the lift that lifts at one corner, the vending machine that only ever has the orange crisps left, the chair by the window where I learned to sleep sitting up. My daughter was born with esophageal atresia, which is a long way of saying the tube that should carry food from her mouth to her stomach was never finished, came into the world in two pieces that did not meet. She is one of roughly one in four thousand. [Logos] I used to find that number comforting and then I stopped, because a number does not feed anybody.

2 What they do not tell you, in the early days, is how much of this is waiting. There is the surgery, yes, and there were several of them, and there is the gastrostomy tube, the little port in her belly through which she has taken nearly every meal of her life. I got fast at it. I can prime a feed in the dark, half asleep, the way you used to know the words to a song. [Simile] What I never got used to was the watching: watching her watch other children eat, watching her learn that her hunger and her stomach were strangers to each other.

3 And there is a longer dread underneath the daily one. When the doctors first talked, carefully, about what a transplant might one day mean, they also said the word that has lived in me ever since: immunosuppression. Anti-rejection drugs, for life. Drugs that would leave her more open to every passing infection, and that carry, the literature is plain about this, a two to four fold higher risk of cancer down the years. You trade one fear for another and you call it hope. I signed forms I did not fully understand. I would sign them again.

4 So here is what it is to read, in a corridor I have memorized, that a team at Great Ormond Street and University College London has grown an esophagus in a laboratory, two and a half centimeters of it, from a pig stripped of its own cells and reseeded with the recipient's, so that the body would not need to be drugged into accepting it, and that in eight small pigs the thing took, grew its own muscle and nerves and blood, let them swallow and grow at a normal rate, with no rejection because there was nothing foreign left to reject. [Cumulative and periodic sentences] Five of the eight were still well at six months. The work was funded by a hospital charity and published in March. [[4]]I read it twice on my phone and then I put the phone face down on the windowsill.

5 I want to be careful with hope; I have been burned by it. The scientists themselves say human trials could be five years off, and five years is a geological age in this corridor. Some of the lab animals developed the same scar-tissue narrowing my daughter knows by name, and it had to be opened up again, the way hers has. [Concession] So I am not telling you my child is saved. I am telling you that for one evening the trade went away. For one evening I imagined a version of her who eats because she is hungry, who does not carry a pharmacy inside her own immune system, and I let myself sit in that room.

6 She is asleep now, two tiles down, the feed running quiet. I do not know what her life will be. But somewhere a pig grew a throat that was its own, and a stranger I will never meet signed the grant that paid for it, and that is closer than we were on Friday. [Polysyndeton] I will be in this chair when she wakes.

Devices, and why they are here:

Logos (paragraph 1): “She is one of roughly one in four thousand.”

The parent reaches for the incidence figure, roughly one in four thousand, to locate a private grief inside a real scale; the writer then immediately undercuts it (a number does not feed anybody), which is why the appeal lands as honest rather than clinical. Choosing statistical evidence here, and then refusing to be consoled by it, is the move.

Simile (paragraph 2): “I can prime a feed in the dark, half asleep, the way you used to know the words to a song.”

The explicit comparison (priming a night feed the way you used to know the words to a song) makes a clinical routine intimate and bodily, turning competence into something the reader can feel rather than just understand. The like/as construction marks it as simile, not metaphor.

Cumulative and periodic sentences (paragraph 4): “So here is what it is to read, in a corridor I have memorized, that a team at Great Ormond Street and University College London has grown an esophagus in a laboratory, two and a half centimeters of it, from a pig stripped of its own cells and reseeded with the recipient's, so that the body would not need to be drugged into accepting it, and that in eight small pigs the thing took, grew its own muscle and nerves and blood, let them swallow and grow at a normal rate, with no rejection because there was nothing foreign left to reject.”

One long looping sentence front-loads the news and then keeps adding clauses (the pig, the cells, the swallowing, the growth, the no-rejection) so the reader experiences the breakthrough the way the parent does, as a rush of detail arriving all at once in a place built for waiting. The loose, accumulating shape is the emotional payload.

Imagery (paragraph 4): “”

The small concrete action, putting the phone face down on the windowsill, shows the weight of the news landing on the body without naming any emotion. The sensory detail does the work an adjective would do in a weaker register.

Concession (paragraph 5): “Some of the lab animals developed the same scar-tissue narrowing my daughter knows by name, and it had to be opened up again, the way hers has.”

The parent grants the bad news its full force, that some animals developed the same scar-tissue narrowing the daughter already lives with, before allowing any hope back in. Conceding the limit plainly is what earns the trust; it keeps the piece from reading as wishful.

Polysyndeton (paragraph 6): “But somewhere a pig grew a throat that was its own, and a stranger I will never meet signed the grant that paid for it, and that is closer than we were on Friday.”

The piled and...and...and slows the closing sentence and gives each clause its own weight, so the pig's own throat, the unknown donor, and the gain since Friday accumulate into a quiet, earned lift rather than a tidy summary.

AP-style multiple choice:

1. In the first paragraph, the writer cites the figure of roughly one in four thousand and then says she stopped finding it comforting because a number does not feed anybody. This sequence primarily serves to
 - A) introduce the scientific breakthrough that the rest of the opener will explain
 - B) prove that esophageal atresia is rarer than readers would assume and is therefore underfunded
 - C) argue that medical statistics are generally unreliable and should be distrusted by patients
 - D) establish that the writer has lived the situation while signaling that statistics fall short of the daily reality she describes
2. Which of the following best characterizes the writer's tone toward the breakthrough across the final three paragraphs?
 - A) detached and analytical, weighing the research as a neutral observer would
 - B) bitter and resentful that the treatment did not arrive sooner for her child
 - C) triumphant and certain that her daughter's condition will soon be cured
 - D) guarded and self-protective, allowing herself hope only briefly and on stated conditions

Multiple choice answer key:

1. Answer D. The writer pairs the incidence figure with her own refusal to be consoled by it, which both grounds her authority in lived experience and marks the gap between data and a child's hunger. The option about rarity and underfunding imports a funding claim the opener never makes (out-of-passage import). The option about statistics being unreliable inflates one personal moment into a sweeping distrust the text does not support (over-broad or extreme). The option about introducing the breakthrough names a real later purpose but ties it to the wrong span, since the breakthrough is not raised until the fourth paragraph (right effect, wrong location).

2. Answer D. The writer says she wants to be careful with hope, notes the five-year horizon and the recurrence of strictures, and permits herself the better future only for one evening, which is hope held under guard. The triumphant-and-certain option contradicts her explicit refusal to say her child is saved (inverted relationship). The detached-and-analytical option ignores the intensely first-person stake and misreads the register as neutral reportage (right device, wrong effect). The bitter-and-resentful option mistakes wariness for resentment, a stance the text never voices (true but not the function asked).

Discussion questions:

1. The writer says of the immunosuppression decision, You trade one fear for another and you call it hope. How does that line shape the way you read her response to the new research later in the opener, and what does it reveal about what hope costs her?
2. This opener is written entirely from one parent's point of view, with no doctor or scientist given their own voice. What does the first-person limitation gain for the piece, and what might a more reported version include that this one leaves out?

Sample strong discussion responses:

1. The trade line teaches us to distrust easy uplift, so when the breakthrough arrives the reader is already braced. Because the parent has told us her previous hope came bundled with a new fear (the cancer risk, the lifelong drugs), we understand why she puts the phone face down instead of celebrating. The research matters to her precisely because it promises a hope with no hidden trade, a swallow with no pharmacy inside the immune system. The line reveals that for her, hope has never been free; it has always been a fear she agreed to carry. That is why one evening without the trade is worth a whole paragraph.

2. The single point of view gains intimacy and authority a reported piece could not match: we believe her because she has memorized the corridor, primed feeds in the dark, signed the forms. Her knowledge is bodily, not borrowed. What it leaves out is proportion and certainty. We never hear a scientist qualify the animal results, never learn how representative eight pigs are, never get the counterweight about how far a large-animal model sits from a human child. The opener is honest about this (she names the five-year horizon and the strictures herself), but the limitation means we feel the news more than we can weigh it. That trade, feeling over weighing, is the register's whole bet.

Writing prompt (Homework or extended in-class, Q2 rhetorical analysis):

The following passage is a first-person opener written in the voice of a parent whose child was born with esophageal atresia, responding to news of a laboratory-grown esophagus that may one day be transplanted without anti-rejection drugs. Read the passage carefully. Then write an essay that analyzes the rhetorical choices the writer makes to convey her complicated response to the breakthrough. In your response you should develop your analysis with specific, well-chosen evidence from the passage; consider how choices such as the writer's concrete imagery, sentence structure, handling of statistics, and concessions shape the reader's understanding of her guarded hope; and explain how these choices serve her purpose. Avoid mere summary or simple device-spotting; tie each choice to its effect on the reader.

Common misconceptions to watch for:

- Students may read the long fourth-paragraph sentence as a run-on error rather than a deliberate cumulative sentence. Point them to the marked device: the piling of clauses enacts the rush of news arriving in a place built for waiting, and the shape is a choice, not a mistake.
- Students may label the night-feed comparison a metaphor. Because it uses the way you used to know the words to a song, it is an explicit comparison and therefore a simile; the marked device flags this look-alike trap directly.
- Students may treat the persona as the literal author and assume every detail is verified autobiography. The science (the incidence figure, the immunosuppression risks, the lab results) is real and sourced, but the parent and child are a constructed first-person voice; the lived details build trust, they are not a biographical claim.
- Students may read the opener as straightforwardly hopeful and miss the guardedness. The concession about the strictures and the repeated insistence I am not telling you my child is saved do the real tonal work; hope here is rationed, not declared.

AP exam alignment:

This opener teaches how a first-person narrative builds meaning by implication rather than assertion, drawing on CED Big Idea 2 (Claims and Evidence) and Big Idea 4 (Style). Students can analyze how concrete personal detail, a cumulative sentence, a simile, and a frank concession control tone and earn credibility, and how an author rations hope instead of declaring it. The pair of questions samples one Claims-and-Evidence reasoning skill and one Style tone skill, and the FRQ is a Q2 rhetorical analysis, the fit for an experience-driven passage that renders a moment rather than arguing a contestable proposition.